

System Design

Standard:

Define the design of the system and how the organizations integrate to form one System of Care.

Purpose:

The ATCEMS System is comprised of multiple agencies that include a diverse group of healthcare professionals including Communications Specialists, First Responders, Transport Providers, Hospital Networks (including specialty receiving centers) and Physicians with varying specialties in the community. Together, this "System" provides the basis for seamless delivery of care to acutely ill or injured patients in our community.

Application:

The ATCEMS System maximizes the opportunity to deliver appropriate care to patients as defined by the Guidelines, Procedures and Standards established by the OCMO (Collectively the Clinical Operating Guidelines). The goal of these documents is to provide safe consistent and sophisticated care to the citizens and visitors of the City of Austin and Travis County.

Medical Direction for all EMS Providers and First Responders flows from the Office of the Chief Medical Officer to each Texas Department of State Health Services (DSHS) Licensed System Organization, via Provider and First Responder Organization Agreements. In order for Medical Direction to flow from the Licensed Organization to the System Credentialed Providers they must respond and provide patient care with the approval of their Licensed Organization (s) within the State of Texas only.

Should they provide COG level patient care at preplanned events not approved by their System Licensed Organization and/or outside the State of Texas; System Medical Direction does not apply. This provision does not preclude Providers from "stopping to render first aid".

1. All medical care within the EMS System should be provided in accordance with the current Clinical Operating Guidelines; by individuals currently certified/licensed by the Texas DSHS and credentialed by the OCMO.
2. Individuals holding current Qualifications may deliver specialty care as defined by the COGs when appropriate equipment and conditions exist.
3. All organizations providing medical care as part of the EMS System will comply with Texas Department of State Health Services requirements for Provider or First Responder Organization Licensure.
4. All 9-1-1 requests for care will be managed by EMS Communications according to the requirements of the currently adopted Medical Priority Dispatch System. This includes call triage, pre-arrival instructions and response determinants.
5. All Tier 2 First Response Organizations will be capable of delivering, at a minimum, Basic Life Support care (PL2) as defined by the OCMO.
6. First Response PL3 – PL5 level of care is supplemental to the System minimum requirements.
7. All System First Response Organizations must maintain the BLS supplies identified on the Minimal Equipment List. If a System Registered Organization chooses to equip a PL3 – PL5 Credentialed Provider, the equipment must be supplied and maintained according to the appropriate Minimal Equipment List for that level of care.
8. Standby and on-site Special Event Providers Minimal Equipment will be determined based on the need of the specific event.

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9. Treatment of patients with prescription or non-prescription medications that are not included in the COG or not approved by OLMC is considered practicing outside the provider's scope of practice.
10. The use of Equipment or Supplies not approved by the System Medical Director during patient care is prohibited. Approved items are specified per the Equipment Lists for each System Organization Tier and Credentialing Level. Refer to OCMO References and current Transport Unit MELs
11. During unusual or extreme conditions or circumstances, the above criteria may be modified by the Medical Director to best meet the needs of the patients of the EMS System.